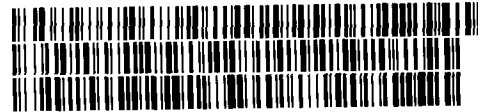


UUID: B52E150F-5B34-482A-84E7-1739B9AC09B7
TCGA-AC-A3W6-01A-PR

Redacted

Sex: _____
D.O.B.: _____
MRN #: _____
Ref Physician: _____



SPECIMEN INFO

Collected: _____
Received: _____
Reported: _____

SURGICAL PATHOLOGY REPORT

DIAGNOSIS

DIAGNOSIS:

Right modified radical mastectomy with sentinel node mapping:

Invasive lobular carcinoma.

Lobular carcinoma was previously confirmed by negative e cadherin expression.

Size: 5.3 cm.

Architectural score: 3 of 3.

Nuclear score: 2 of 3.

Mitotic score: 1 of 3.

Total score: 6 of 9 = grade 2.

Carcinoma involves sections of the nipple but does not involve the overlying epidermis and is not seen in dermal lymphatics.

Prognostic panel was performed on the previous biopsy and will not be repeated unless requested.

Carcinoma does not involve inked deep margin of excision.

Right sentinel lymph nodes:

Three lymph nodes, metastatic carcinoma in three of three nodes.

Size of involvement within the node: 1 cm.

Extracapsular extension of carcinoma present.

Confirmed by staining with pancytokeratin.

TNM: T3N1MX.

1 CD-0-3
carcinoma, lobular, infiltrating
8520/3

Site: breast, NOS
C50.9

Electronic Signature:

5-1-12
RD

CLINICAL INFORMATION

CLINICAL HISTORY:

Preoperative Diagnosis: Right modified radical mastectomy with sentinel node mapping with frozen section. Invasive mammary carcinoma, ER positive, PR positive.

Postoperative Diagnosis:

Symptoms/Radiologic Findings:

SPECIMENS:

Right breast, tie marks sentinel node

SPECIMEN DATA

GROSS DESCRIPTION:

The specimen is received in a single formalin-filled container labeled with the patient's name 'right breast, tie marks sentinel node' and consists of a 281.8 gram, 20.0 x 11.5 x 3.8 cm mastectomy specimen partially surfaced by a 15.8 x 7.5 cm ellipse of pink-tan wrinkled skin bearing an eccentric 0.9 x 0.8 cm inverted nipple. A suture is present at the lateral edge and denotes the sentinel lymph node. A discrete axillary tail is not identified. The deep margin is inked and the specimen is serially sectioned to reveal a 5.3 x 3.5 x 3.3 cm ill-defined gray-white mass originating centrally subjacent to the nipple and abutting the skin surface at the nipple. This mass approaches to within 0.7 cm of the inked deep margin. The uninvolved tissue is comprised of yellow-tan adipose tissue admixed with minimal amounts of interspersed gray-white fibrous tissue. Focal areas of nodularity are present at the medial and lateral borders, however, discrete invasive mass is not identified. The closest of these nodules approaches to within 1.5 cm of the inked margin.

Adjacent to the aforementioned suture, denotes a 2.0 cm well delineated nodule consistent with lymph node. Within the adjacent fibroadipose tissue are two additional firm fatty nodules consistent with lymph node, 0.5 and 1.5 cm in greatest dimension.

Representative sections are submitted in cassettes 1 through 12 labeled designated as follows: 1 and 2, mass to include nipple and adjacent skin; 3 and 4, mass to inked deep margin, perpendicular; 5 and 6, additional mass; 7, upper outer quadrant; 8, lower outer quadrant; 9, upper inner quadrant; 10, lower inner quadrant; 11, one whole possible sentinel lymph node, bisected; 12, two additional lymph nodes (one inked and bisected). Additionally, a yellow, green and blue cassette are submitted for genomics research each labeled

criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
IHPAA Discrepancy		X
Prior Malignancy History		X
Local/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed/Initials	RM	
Date Reviewed	4/25/12	